

U.S. Department of State / GHSD — Advancing Global Health (DFOP0017890)

Statement of Interest — Phase 1

FairBanks Medical Centre · FairBanks Community Reach · FCHIP

Track 2, Objective 4: Strengthen and Scale Interoperable, Fit-for-Purpose Digital Health Systems

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Table Listing of Critical Details

Item	Detail
Proposed Project Title	Connecting Community Health Visits to Uganda's National Digital Health Systems (FCHIP)
Name of the Organization	FairBanks Medical Centre
Addendum to which the SOI is responding	Addendum E: Health Foreign Assistance Memorandum of Understanding (MOU) Implementation in Uganda
Target Benefiting Country	Uganda
Total Federal Share Requested	\$1,850,000
Total Cost Share (as applicable)	\$0 (none proposed at this stage)
Project Length	36 months

1. Issue / Challenge / Opportunity

Uganda is moving through a five-year health transition (April 2026 to December 2030). Clinics, districts, and communities must keep HIV, TB, malaria, maternal and child health, immunisation, and outbreak services running well while more work shifts to government-led programmes. That only works if leaders can see what is happening in communities in time to act - not weeks later from paper books.

Community Health Workers and Village Health Teams already visit homes and send people for care. Too often, what they record never reaches eCHIS, clinic records, DHIS2/eIDSR, or the National Data Warehouse in a form people can use. Then national systems miss rising fever cases, missed antenatal visits, gaps in immunisation, and medicine pressure. Care stays reactive, follow-up is weak, and hard-won gains can slip.

There is a clear chance to fix this: help community visit data flow into Uganda's National Digital Health Architecture (NDHA) so Ministry of Health systems are fuller and more useful, and so government can take over step by step - without building a separate parallel system.

2. Proposed Solution / Activities

FairBanks Medical Centre and FairBanks Community Reach submit this Statement of Interest under Track 2, Objective 4 of Addendum E: strengthen and scale digital health systems that work together, fit Uganda's needs, and move toward government ownership and steady day-to-day running.

We want to trial, then hand over, a practical tool - the FairBanks Community Health Intelligence Platform (FCHIP) - that lets CHWs and VHTs record household and visit information on phones even offline, sync it safely, match it to national reporting needs, and help the clinic and district act. FCHIP is not meant to replace Uganda's national systems or duplicate existing U.S.-supported digital health work. It is a complementary bridge so community work shows up where it should and helps prevent problems earlier - without creating a parallel silo.

We deliberately focus on a feasible subset of Objective 4 systems for our catchments: household and visit data in line with eCHIS; referral and visit handoff into clinic systems (such as eAfya or ClinicMaster where used); summary reporting into DHIS2 and alerts that support eIDSR; readiness to feed the National Data Warehouse; and CHW/VHT workforce lists that can align with iHRIS where the rules allow. Laboratory, warehouse, and other named national systems stay under Ministry and partner lead; we coordinate rather than rebuild them. We will follow NDHA and OpenHIE-style sharing rules, under Ministry of Health guidance.

We want every partner to gain something lasting. No one should depend forever on a private tool that sits outside government systems.

Who gains	What they gain
Uganda / Ministry of Health	More complete community data in eCHIS, DHIS2/eIDSR, and related national systems; clearer proof that referrals were closed; and documented transition of staffing and recurrent operating costs to MoH by project close.
U.S. Department of State / GHSD	A Ugandan partner with a working clinic and community network that keeps essential services going while showing digital systems can connect under the MOU - in ways that can be measured and handed on, supporting U.S. goals around health security and country self-reliance.
Districts & facilities	Simple screens to plan outreach, follow referrals, and track programmes; maps that show hotspots so teams can act before outbreaks hit the ward.
Communities & CHWs/VHTs	Easy tools that work offline, quicker support from the medical centre, and care that reaches mothers, children, older people, and people with long-term illness earlier.
FairBanks / FCHIP	A real place to test and write down a working data-sharing approach, then share the guides, training, and handoff plans with Ministry-aligned partners.

What we will do:

- A1. Put simple offline phone tools in the hands of CHWs and VHTs so household and visit records match eCHIS ideas across FairBanks Community Reach areas.
- A2. Set up safe syncing and clear access rules so community records reach FairBanks Medical Centre work and authorised district viewers, in line with Uganda's data protection and security expectations.
- A3. Match community indicators to DHIS2 reports and show how summary data can move for maternal and child health, immunisation, chronic disease screening, and disease watch (including eIDSR-related alerts).
- A4. Give clinics and programmes clear dashboards and hotspot maps that help trigger outreach, referrals, and supply planning.
- A5. Write the sharing rules, data-handling guides, training materials, and a documented transition plan for staffing and recurrent operating costs to the Ministry of Health by project close - ready for a full proposal if we are invited.

How we will work:

We start from how FairBanks already works: communities name needs; CHWs and VHTs visit, teach, and refer; FairBanks Community Reach runs outreach and home visits; FairBanks Medical Centre provides clinic care, tests, pharmacy, and referrals; FCHIP helps the information move back and forth so teams can act.

We will build for sharing, not for locking data away. Priority links are eCHIS, DHIS2/eIDSR, clinic record systems (eAfya or ClinicMaster where used), and the National Data Warehouse when the national formats are confirmed. We will coordinate with incumbent digital health investments so FCHIP feeds government systems rather than competing with them.

Digital work must support, not distract from, life-saving care. Community forms will include HIV, TB, and malaria flags where useful, plus maternal and child pathways, immunisation follow-up, and early outbreak warning.

We will shape forms and alerts with CHWs, clinic staff, and local leaders; train Ugandan staff who will keep the system going; and write every link clearly for Ministry of Health review, including how progressive GOU responsibility, staffing, and recurrent costs move off the award by close.

We start where FairBanks already works - Bukoto, Kyebando, Kisaasi, Kamwokya, Kikaaya and nearby communities - and design so other districts with CHW/VHT networks can copy the approach later.

3. Anticipated Outcomes and Results

Success advances the Department of State America First Global Health Strategy: save lives, strengthen health systems, enhance efficiency, foster self-reliance, and protect U.S. health investments that support American safety, strength, and prosperity:

Outcome	What success looks like
Save lives	Earlier notice of missed antenatal visits, immunisation gaps, fever clusters, and lost referrals - followed by CHW action and clinic support.
Strengthen systems	Working links and sample exports for eCHIS-style records and DHIS2/eIDSR summaries, with clear guides for the Ministry and partners.
Work more efficiently	Less double entry and less waiting on paper; screens that help teams move from a community signal to clinic or district action faster.
Build self-reliance	Documented transition of staffing and recurrent operating costs to the Ministry of Health by project close, with guides, training, and hosting options that Ugandan teams can run.
Support U.S. interest	A clear contribution to the MOU digital shift: community data that connects under government authority, protects past U.S. health investments, and strengthens early warning for outbreaks.

4. Organizational Capacity

- We are a Ugandan organisation with a working medical centre - clinic care, tests, pharmacy, and referrals already running every day.
- FairBanks Community Reach is active in maternal and child health, Gericare, chronic disease screening, and school and workplace outreach.
- We already work with CHWs and VHTs in the Kampala-area communities named above. This is not starting from zero.
- We already keep digital clinic records and pharmacy dispensing data that support day-to-day facility work.

- Community Health Insurance (CHIS) and livelihood links already run inside FairBanks Community Reach - useful context for sustainable community financing, though this SOI asks only for Objective 4 digital interoperability work.
- Research, ethics, and skills training are part of how FairBanks works with communities and partners.

5. How Systems Will Connect (chart)

System	How we connect
eCHIS	Household lists, visit logs, and referral flags from phone tools that work offline
Clinic records (eAfya / ClinicMaster)	Referral handoff and visit links at FairBanks Medical Centre
DHIS2 / eIDSR	Summary reports and disease alerts for maternal/child health, immunisation, outbreaks
National Data Warehouse	Standard data feeds when the national format is confirmed
iHRIS	CHW/VHT worker lists aligned where the national rules allow

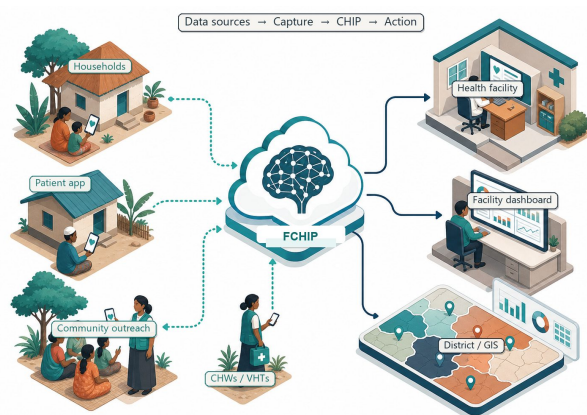


Figure 1. From community visits through FCHIP into national digital health systems (chart; not counted in the page limit).

6. Alignment with Addendum E Guiding Principles (chart)

Principle	How FairBanks responds
Sustainability & country ownership	Build with MoH/NDHA rules and plan a clear handoff over time
Protect essential services	Digital tools support HIV, TB, malaria, maternal/child care, and immunisation
Data for action & accountability	Good data into government systems that people actually use to decide
Integrated, people-centred care	One community-to-clinic loop; less fragmented work
Local partnerships	Government, CHWs/VHTs, community groups, schools, and NGO partners
Evidence-based & context fit	Try in real communities; measure; improve before wider use

Principle	How FairBanks responds
Oversight & coordination	Stay aligned with Ministry priorities and joint review meetings

7. Partner Roles (early list)

The partners below are coordination partners for Phase 1, not proposed subrecipients. FairBanks Medical Centre is the sole lead applicant; no federal share is allocated to partners at this stage. If Phase 2 or joint design proposes subawards, we will provide organisation names, points of contact (phone and email), federal share, cost share, and refined roles then.

Partner	Proposed role
Ministry of Health (digital health / community health)	Align with NDHA, eCHIS, and national reporting; review links and the handoff plan
District health teams (pilot areas)	Check summary reports, use dashboards, and discuss readiness
CHWs, VHTs, and community leaders	Main partners for data; help shape forms and alerts
Academic partners (to be confirmed later)	Help with learning and testing how systems share data
NGOs / community groups working with FairBanks Community Reach	Help coordinate outreach and community work

8. Resource Contributions and/or Cost Share

We are not proposing cost share at this stage. FairBanks will keep running its medical centre and community outreach as normal. If invited to the next stage, we can discuss any in-kind support or matching help then.

9. Readiness for Phase 2

With this Statement of Interest, FairBanks asks to move Track 2, Objective 4 forward through a digital health approach rooted in community work, built to connect with government systems, and designed for Ugandan ownership. We hope to be invited to Phase 2 (full proposal) or joint programme design, where we will set out workplans, budgets, monitoring, partner letters, and a clear plan to hand over staffing and running costs.

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